



**HIKE HEALTH®**

Health Information Knowledge  
Exchange

EDITION 2026 · A FIELD GUIDE FOR PAYERS

THE INTEROPERABILITY OPERATING MODEL · BEHAVIORAL HEALTH EDITION

# The payer field guide.

An executive briefing on CMS regulations, digital quality measures, and the operating model that turns API readiness into business value, tailored for Philadelphia's behavioral health Medicaid managed care organization.

CMS-9115-F · CMS-0057-F · CMS-0053-F · CMS-4208-F2 · CMS-0062-P · NCQA dQM · 42 CFR PART 2

PREPARED FOR

## Community Behavioral Health (CBH)

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PART I The Climate

01 • EXECUTIVE SUMMARY

# One operating model. ~780,000 Philadelphians. Material business impact.

CBH is the sole behavioral health MCO for Philadelphia County under Pennsylvania's HealthChoices program, covering roughly 780,000 Medicaid-eligible residents on behalf of the City and DBHIDS. The CMS interoperability rules that apply to Medicaid managed care plans, NCQA's digital quality transition, and 42 CFR Part 2 consent all stand on the same FHIR and HL7 foundation. **Build once. Operate continuously.** The organizations that create the most value turn regulatory investment into a reusable enterprise capability.

**3+1**

**RULES THAT BIND CBH**

9115-F, 0057-F, 0053-F apply directly; NCQA dQM follows the accreditation CBH already holds

**1/1/2027**

**API GO-LIVE**

Prior Auth, Provider Access & Payer-to-Payer APIs, first rating period on or after this date

**~780K**

**MEMBERS SERVED**

More than 40% of Philadelphia; every workflow improvement lands at population scale

## What the CBH reader will take away

**INSIGHT 01**

### One stack, every rule

The CMS rules that bind CBH and NCQA dQM share FHIR R4, USCDI, and HL7. A single foundation services all of them, and the behavioral health data CBH stewards.

**INSIGHT 02**

### API readiness is not enough

Compliance is an operating program, not a one-time build. For a BH-MCO the run-state includes Part 2 consent and a provider network that still runs on fax.

**INSIGHT 03**

### Operating model decides cost

A foundational FHIR server compounds across every rule and downstream use case, including the BH quality measures CBH reports today. Facades narrow the option set.

The payers that finish on schedule treat the regulations and dQM as one operating-model upgrade, not parallel scrambles. For CBH, the upgrade also has to respect what makes behavioral health different: Part 2 consent, community providers, and members in active treatment.

HIKE HEALTH® • ANALYSIS



## 02 · WHAT'S AT STAKE FOR CBH

# The cost of treating this as a project, not an operating model.

Each regulation carries operational risk well beyond the deadline, and in behavioral health the failure modes are more visible, not less. The risks compound when programs run in parallel without a shared foundation.

### RISK 01 · REGULATORY

#### Audit exposure on annual filings

ePA metrics, USCDI completeness, and API uptime are now annual obligations, with PA DHS/OMHSAS and CMS both watching. Gaps surface publicly and during audits of the HealthChoices contract.

### RISK 02 · FINANCIAL

#### Vendor sprawl and duplicate spend

Separate vendors for each rule means duplicated FHIR work, integration debt, and a stack no one owns end-to-end, on a nonprofit's administrative budget, funded by public dollars.

### RISK 03 · PROVIDER & MEMBER EXPERIENCE

#### Provider abrasion in a stretched network

Prior auth turnaround for inpatient psych, residential SUD, IOP, and ABA is the most visible failure mode in a community provider network already under workforce strain. Members in crisis feel every delay.

### RISK 04 · STRATEGIC

#### Locked out of AI and dQM use cases

If CBH's FHIR data is not retained and queryable, the dQM and care-coordination roadmap stalls, and the leverage from compliance spend disappears.

### THE CBH LENS · 42 CFR PART 2

Behavioral health adds a risk no physical-health payer carries at this intensity: **substance use disorder records move only with consent under 42 CFR Part 2**. Patient Access, Payer-to-Payer, and Provider Access APIs all touch Part 2 data. Consent management is not a compliance afterthought here; it is a first-class architectural requirement.

## What “good” looks like

### OPERATIONAL

#### One backlog, not five

A single program with one accountable team across regulations, vendors, and reporting to CMS, DHS, and DBHIDS.

### ARCHITECTURAL

#### A durable FHIR foundation

USCDI-aligned, queryable, consent-aware, shared across Patient Access, ePA, and dQM.

### STRATEGIC

#### Compliance that compounds

Each rule's spend extends to the next, and to the quality and care-management use cases beyond.



PART II CMS Regulations

03 • THE REGULATORY LANDSCAPE

## What applies to CBH. What doesn't. And the one timeline that matters.

CBH is a Medicaid managed care plan, not a Medicare Advantage organization, and not a pharmacy benefit. That changes the applicability map, but not the foundation: FHIR R4, USCDI, and HL7 standards service every rule that binds CBH.

THE CLIMB • CBH REGULATORY TIMELINE • 2021 – 2030



† Medicaid managed care dates apply by the first rating period beginning on or after January 1 of the year shown.

| RULE        | APPLIES TO CBH? | BUSINESS CONSEQUENCE FOR CBH                                                                                                                                       |
|-------------|-----------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| CMS-9115-F  | YES             | <b>Foundation.</b> Patient Access & Provider Directory APIs for Medicaid managed care, live today via HealthTrio (an mPulse company). Every later rule extends it. |
| CMS-0057-F  | YES             | <b>The deadline.</b> ePA APIs and public metrics. Provider abrasion drops; turnaround goes on display. Rating-period dates.                                        |
| CMS-0053-F  | YES             | <b>Fax dies.</b> HIPAA attachments standard hits BH's most fax-dependent workflows: medical necessity & clinical documentation.                                    |
| CMS-4208-F2 | NO • MA ONLY    | Plan Finder submission binds MA plans. CBH's directory duty runs through Medicaid managed care rules & HealthChoices (see p. 9).                                   |
| CMS-0062-P  | WATCH           | Drug ePA targets Part D & pharmacy benefit. In PA HealthChoices, pharmacy sits outside CBH, but BH medication workflows intersect (see p. 9).                      |
| NCQA dQM    | YES             | CBH is NCQA-accredited. The dQM transition converts BH quality reporting from chart-chase to continuous, on the same stack.                                        |



04 · CMS-9115-F · THE FOUNDATION

## Patient access. The baseline every rule extends, with a Part 2 overlay.

9115-F gives CBH members a right to their behavioral health data and providers a way to find network. Done well, it becomes the foundation for every later rule. Done badly, it surfaces as portal complaints, audit findings, and consent violations.

Effective now

FHIR R4

USCDI v3

SMART on FHIR

42 CFR Part 2 consent

EXHIBIT · THE MEMBER DATA WORKFLOW · BEHAVIORAL HEALTH

Member

**Authenticates**

via member app or third-party tool

**Consents**

granular Part 2 consent for SUD records: who, what, how long

**Receives**

claims, clinical, and coverage data

**Acts**

manages care, shares history with new providers

CBH

**Identity**

IAL2-proofed identity flow

**FHIR R4 store**

retained, USCDI-aligned, consent-segmented

**API delivery**

patient access · provider directory

**Audit log**

access events, consent decisions, reporting metrics

THE CBH LENS

For a BH-MCO, the hard part of Patient Access is not the API; it is **data segmentation**. SUD claims and clinical records must be separable at the resource level so consent can be enforced, not approximated. Payers that bolt consent on after the FHIR build end up rebuilding. The 2024 Part 2 alignment with HIPAA simplified the rules; it did not remove the consent requirement.

BUSINESS OUTCOME

**Member trust and self-service**

Members can move their history without calling Member Services. Portal volume drops; continuity of care across PH and BH improves.

STRATEGIC VALUE

**A reusable foundation**

The consent-aware FHIR store CBH stands up here services 0057-F, 0053-F, and dQM with no rebuild.

**Where CBH stands today.** CBH's Patient Access and Provider Directory APIs run on HealthTrio, an mPulse company, conforming to CARIN Blue Button, US Core, and Da Vinci Plan-Net; CBH's public interoperability page notes that the 0057-F APIs "will be added soon." With CBH's core systems on-premises, every public API has to be hosted somewhere; today that somewhere belongs to the vendor. That makes 2026 the decision point: extend the portal vendor's facade into ePA, or stand up a consent-aware cloud FHIR foundation CBH owns, fed from the on-prem systems of record. HIKE runs that evaluation vendor-agnostically and manages any migration without member disruption.

HIKE HEALTH® · SOURCE: CBHPHILLY.ORG · PORTAL INTEROPERABILITY INFORMATION



05 · CMS-0057-F · THE DEADLINE

## Prior authorization. From days to minutes, reported publicly.

For Medicaid managed care plans, operational provisions bind from the first rating period beginning on or after **January 1, 2026**, and the Prior Authorization, Provider Access, and Payer-to-Payer APIs from the first rating period on or after **January 1, 2027**. ePA metrics are filed annually by March 31 and made public. The rule replaces faxes and portals with FHIR-based decision flows, and puts CBH's turnaround times on display.

APIs · rating period on/after Jan 1, 2027

FHIR Da Vinci PAS

CRD / DTR

USCDI v3

Annual metrics · Mar 31

EXHIBIT · THE EPA WORKFLOW · BEHAVIORAL HEALTH PROVIDER TO DECISION

|                 |                                                                  |                                                                                   |                                                                                       |                                                  |
|-----------------|------------------------------------------------------------------|-----------------------------------------------------------------------------------|---------------------------------------------------------------------------------------|--------------------------------------------------|
| <b>Provider</b> | <b>Orders</b><br>initiates PA in EHR (e.g., IOP after discharge) | <b>Checks coverage</b><br>requirements returned (CRD)                             | <b>Auto-fills</b><br>documentation pre-populated (DTR): ASAM criteria, treatment plan | <b>Submits</b><br>PA sent via FHIR (PAS)         |
| <b>CBH</b>      | <b>Receives</b><br>structured PA submission                      | <b>Evaluates</b><br>auto and manual review path, UM criteria applied consistently | <b>Decides</b><br>expedited 72 hrs · standard 7 days, with specific denial reasons    | <b>Reports</b><br>annual ePA metrics filing      |
| <b>Member</b>   | <b>Sees status</b><br>in real time via portal/app                | <b>Avoids delay</b><br>at the moment of clinical readiness                        | <b>Receives care</b><br>with faster decision turnaround                               | <b>Trust rises</b><br>with transparent timelines |

### Why this rule matters strategically

OPERATIONS

#### Throughput up

Auto-approval paths shrink decision time from days to minutes for routine, criteria-clear requests, freeing UM clinicians for complex cases.

REPUTATION

#### Public ePA metrics

Annual filing makes turnaround visible to regulators, providers, DBHIDS, and the market.

CLINICAL

#### Care at the window of readiness

In behavioral health, a PA delay is not an inconvenience; it can be a lost engagement window. Faster decisions are a clinical outcome.



06 • 0057-F IN BEHAVIORAL HEALTH • THE GROUND TRUTH

## The API is the easy half. Your provider network is the hard half.

CMS-0057-F assumes providers initiate ePA from certified EHRs. Community behavioral health organizations, the backbone of CBH's network, historically sit outside EHR incentive programs. An ePA program that stops at the API will technically comply and practically fail.

### REALITY 01

#### Uneven EHR adoption

BH providers were excluded from Meaningful Use incentives. Many run on BH-specific platforms or paper-adjacent workflows. CRD/DTR adoption will not happen by mandate alone.

### REALITY 02

#### High-touch service mix

Inpatient psych, residential SUD (ASAM-based), ACT, IOP/PHP, and ABA authorizations are concurrent-review-heavy. Structured data pays off fastest exactly here.

### REALITY 03

#### Fax is still the default

Medical-necessity packets move by fax and portal upload today. 0057-F and 0053-F together are the exit ramp, if provider enablement is funded and sequenced.

### THE CBH LENS • WHAT A CREDIBLE EPA PROGRAM INCLUDES

**1.** Segment the PA portfolio: which BH service lines are criteria-clear enough for auto-approval paths (routine IOP concurrent reviews) vs. clinician review (residential, ABA). **2.** Pilot with the top-volume providers first; a small number of BH organizations drive a large share of PA volume. **3.** Offer a low-lift submission path (SMART app or portal-embedded DTR) for providers without capable EHRs. **4.** Instrument metrics from day one; the March 31 filing is also a narrative about CBH's provider experience.

## The two other APIs in the same rule

### PROVIDER ACCESS API • JAN 2027†

#### Network providers pull member data

Treating providers retrieve claims, encounter, and clinical data, with Part 2 consent enforced. Done right, this is CBH's care-coordination backbone with physical health.

### PAYER-TO-PAYER API • JAN 2027†

#### Continuity across plan moves

Members churn between Medicaid plans and counties. P2P exchange preserves treatment history at transitions, the moment BH members are most at risk of falling out of care.

† First rating period beginning on or after January 1, 2027.





07 · THE CARVE-OUT CORNER · CMS-4208-F2 & CMS-0062-P

## Two rules that don't bind CBH directly, and why they still matter.

Deep customization means saying clearly what does **not** apply. CBH is not a Medicare Advantage organization and does not administer the pharmacy benefit. But both rules shape the standards, the vendors, and the expectations CBH will be measured against.

### CMS-4208-F2 · MA provider directories → Plan Finder

- **What it is.** Beginning Plan Year 2027, MA organizations must submit in-network provider data to CMS for publication on Medicare Plan Finder, with attestation and 30-day update duties.
- **Why CBH is out of scope.** The rule binds MA plans. CBH serves Medicaid HealthChoices members in one county with no MA product.
- **Why it still matters.** CBH already owes an accurate, usable directory under Medicaid managed care rules and its HealthChoices contract, and BH network accuracy (who is taking new patients, in which language) is an access-to-care issue, not a back-office task.
- **The reusable asset.** The 9115-F FHIR Provider Directory API is the same infrastructure MA plans are now hardening for public scrutiny. Expect Medicaid to follow where MA leads.

### CMS-0062-P · Drug ePA & updated standards

- **What it is.** Proposed April 2026: extends ePA to drugs via NCPDP SCRIPT, refreshes the Da Vinci PAS IG, sunsets legacy versions October 1, 2027.
- **Why CBH is (mostly) out of scope.** In PA HealthChoices, the pharmacy benefit is administered outside the BH carve-out. CBH is not a Part D sponsor.
- **The intersection CBH owns.** MAT for opioid use disorder, long-acting injectables, and clozapine sit at the seam between pharmacy and BH services. When drug ePA accelerates, CBH's service authorizations must keep pace or become the bottleneck in a member's treatment start.
- **One governance layer.** The IG updates in 0062-P will flow into the same Da Vinci PAS stack CBH builds for 0057-F. Track the standard, not the rule.

#### THE CBH LENS

**Read these two rules as market signals.** They tell CBH where CMS is taking directory transparency and pharmacy automation, and they define the interoperability expectations of the physical-health MCOs, PBMs, and providers CBH coordinates with every day. They also point the same direction operationally: more public, always-on API surface. For an on-prem shop that is a hosting question, and it belongs in the 2026 architecture decision (p. 6), not in a scramble when the next mandate lands.



08 · CMS-0053-F · THE FAX RETIRES

## Claims attachments. The first HIPAA standard, thirty years later.

Published March 24, 2026. Compliance by **May 26, 2028**. The first-ever HIPAA-adopted standards for claims attachments (medical records, clinical notes, treatment plans, lab results), plus electronic signatures. \$782M/yr in projected industry savings. No payer segment is more attachment-dependent than behavioral health.

Compliance · May 26, 2028

X12N 275 / 277 v6020

HL7 C-CDA & Attachments IG

E-signatures

### EXHIBIT · THE CLAIMS ATTACHMENT WORKFLOW

|                 |                                                         |                                                         |                                                                                                |                                                              |
|-----------------|---------------------------------------------------------|---------------------------------------------------------|------------------------------------------------------------------------------------------------|--------------------------------------------------------------|
| <b>Provider</b> | <b>Submits claim</b><br>standard 837 transaction        | <b>Receives request</b><br>277 for additional info      | <b>Bundles docs</b><br>C-CDA clinical attachments (275):<br>treatment plans,<br>progress notes | <b>Signs e-doc</b><br>per the rule's e-signature<br>standard |
| <b>CBH</b>      | <b>Receives</b><br>structured attachments,<br>not faxes | <b>Adjudicates</b><br>with clinical context<br>attached | <b>Decides</b><br>faster, with audit trail<br>intact                                           | <b>Returns</b><br>835 remittance to<br>provider              |
| <b>Outcome</b>  | <b>Fax/mail retires</b><br>manual handling drops        | <b>Cycle shrinks</b><br>days, not weeks                 | <b>Audit-ready</b><br>signed, timestamped,<br>traceable                                        | <b>\$782M/yr</b><br>projected industry<br>savings            |

### Why this rule matters strategically for CBH

#### OPERATIONAL

##### Two years to retire the fax machine

BH medical-necessity documentation is the most fax-bound workflow CBH runs. Clearinghouses and EHRs will move; payers that delay become the bottleneck their providers complain about.

#### STRATEGIC

##### Same plumbing as 9115-F & 0057-F

C-CDA and HL7 attachments reuse the clinical data exchange CBH builds for interoperability, and feed the same store that powers dQM. No parallel stack required.



PART III Digital Quality Measures

09 • THE DQM DIVIDEND

## When compliance becomes leverage.

NCQA's transition to FHIR-based digital quality measures uses the same foundation the CMS rules require. CBH already carries NCQA accreditation, and behavioral health owns some of the most chart-chase-intensive measures in HEDIS. The infrastructure built for compliance pays for itself in quality reporting.

### MY 2030

#### DQM TARGET

Submission path for HEDIS

### FHIR + CQL

#### REQUIRED STACK

Same as 9115-F & 0057-F

### Continuous

#### MEASUREMENT

vs annual chart-chase

#### THE CBH LENS • THE MEASURES THAT MOVE FIRST

The BH measure set is where continuous measurement changes outcomes, not just reporting: **FUH** (follow-up after hospitalization for mental illness), **FUA / FUM** (follow-up after ED visits for SUD / mental illness), **IET** (initiation & engagement of SUD treatment), **AMM** (antidepressant medication management), and **SAA** (adherence to antipsychotics). Every one depends on timely clinical data from providers, the exact data acquisition problem the FHIR foundation solves. In-year visibility on FUH-7 is a care-management intervention, not a report.

## The business case in one line

The FHIR investment that satisfies CMS compliance is the same investment that retires legacy HEDIS chart-chase, and puts follow-up and engagement measures on a dashboard while the member can still be reached.

HIKE HEALTH® • STRATEGY PRACTICE

#### COST

##### Chart-chase retires

Manual abstraction, vendor fees, and seasonal labor decline as measurement moves to FHIR + CQL.

#### QUALITY

##### Real-time visibility

Measure performance is observable continuously, not just at annual submission.

#### STRATEGY

##### AI & care management

A durable FHIR store is the platform for the AI and population-health use cases the next strategic plan will ask for.



10 • FROM CHART-CHASE TO CONTINUOUS MONITORING

## Two operating models. One worth investing in.

The legacy quality stack assumes data is missing and must be collected once a year. The digital quality stack assumes data is present, current, and queryable. For BH follow-up measures, the difference is measured in members re-engaged versus members lost.

### YESTERDAY • MY 2024

#### Legacy HEDIS

Chart-chase plus claims, run annually against a warehouse.

- Manual chart abstraction at submission time, heaviest in BH clinical measures
- Annual snapshot, no in-year visibility on FUH or IET
- Data silos in SQL warehouses
- Limited reuse outside HEDIS

### TOMORROW • MY 2030

#### Digital Quality Measures

FHIR-native, continuous, audit-ready measurement.

- FHIR R4 store, persisted and queryable
- CQL engine executes NCQA logic natively
- Continuous in-year performance visibility, follow-up gaps flagged while actionable
- Same stack as 9115-F and 0057-F

### A four-step transition, sequenced for CBH

#### STEP 01

##### Inventory

Map CBH's HEDIS portfolio to NCQA dQM bundle equivalents; flag the BH measures with the highest abstraction cost.

#### STEP 02

##### Bridge

Stand up FHIR R4 + CQL alongside existing analytics, with no rip-and-replace of the current HEDIS pipeline.

#### STEP 03

##### Pilot

Run parallel reporting on 3–5 high-volume BH measures, FUH and IET first.

#### STEP 04

##### Scale

Migrate the full portfolio as bundles mature; wire in-year results into care management.



PART IV The HIKE Approach

11 • WHY HIKE HEALTH

## Interoperability that survives go-live.

HIKE HEALTH® is the operational layer above your FHIR stack. We are not a software platform vendor, a generic IT consultancy, or a one-time compliance shop. We are your interoperability department, as a service, headquartered in Philadelphia, blocks from CBH.

### ADVISORY

#### Strategy & regulatory readiness

Program design across CMS-9115-F, 0057-F, 0053-F, and NCQA dQM, with the Medicaid managed care and Part 2 nuances a BH-MCO carries. Architecture decisions framed for executive sponsors.

### EXECUTION

#### Professional services

FHIR build, integration, ePA workflow rollout, consent management, identity, member experience, and end-to-end testing.

### OPERATIONS

#### Managed run-state

Ongoing operations with clear SLAs across your FHIR servers, integration engines, and vendors, through every annual filing.

### CONVENER

#### Ecosystem leverage

We bring payers, providers, vendors, and standards bodies to the same table, and keep them there. In BH, provider enablement is half the program.

### What we are not

- A software platform vendor
- A generic IT consultancy
- A one-time compliance vendor
- A temporary implementation shop

### What we bring

- HL7 Da Vinci & FAST contributors
- HITRUST e1 certified delivery
- Vendor-agnostic posture by design
- Payer-native team, with Medicaid and behavioral health depth, not generalists

### THE CBH LENS

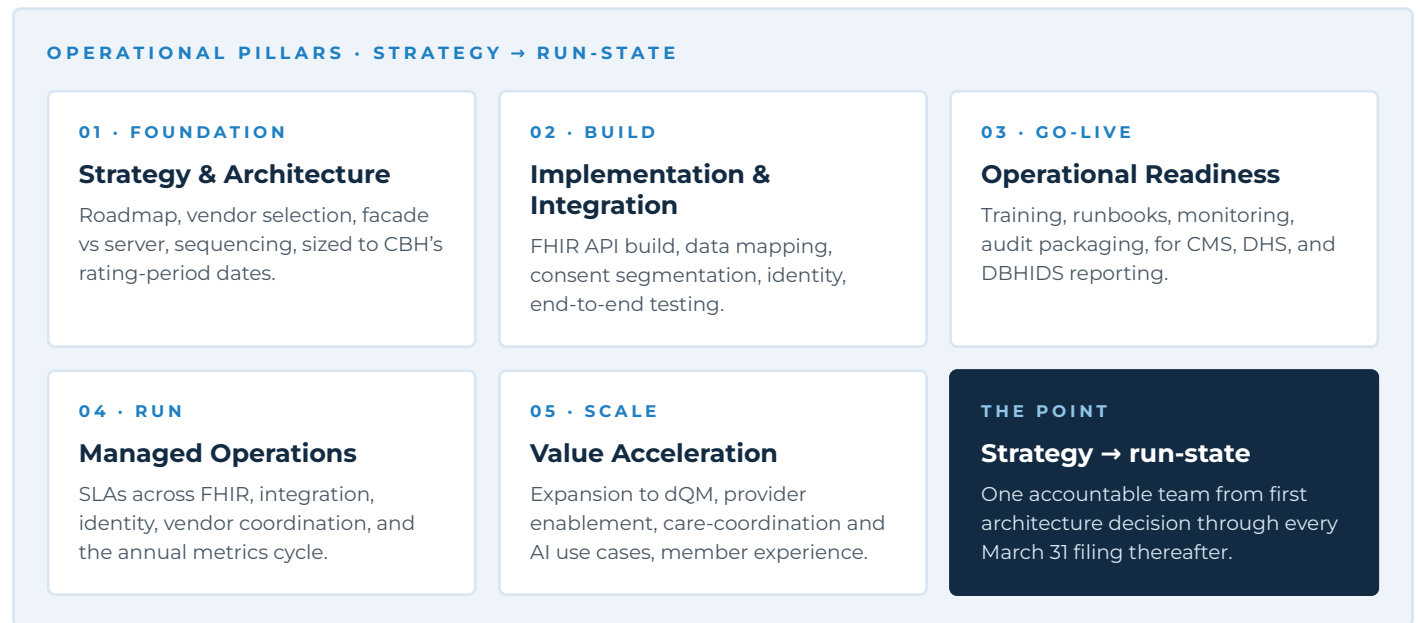
**Same city, same standards bodies, same mission orientation.** HIKE HEALTH works from Philadelphia and builds for public-mission payers. A HealthChoices BH-MCO's constraints (public accountability, nonprofit economics, county partnership) are the context we design for, not an exception we accommodate.



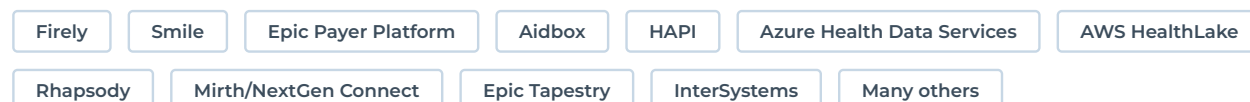
12 • HIKE BASECAMP™

## Five operational pillars. One accountable team.

Basecamp™ is HIKE HEALTH®'s interoperability-department-as-a-service. Each pillar covers a distinct operating responsibility; together they cover the full lifecycle from strategy to managed run-state.



### Vendor ecosystem • HIKE operates across





13 • HOW HIKE HEALTH FITS

## The operating layer above your stack.

HIKE HEALTH® sits above the platforms CBH already owns, coordinating governance, integration, and run-state across your vendors. No rip-and-replace.

### EXHIBIT • THE THREE-TIER OPERATING MODEL

#### LAYER 01

##### HIKE Layer

Governance • orchestration • QA

Governance    Orchestration    QA    Terminology    Reporting  
Data Quality    Identity    Consent • Part 2    Vendor Coordination

#### LAYER 02

##### Your Platforms

Clinical & admin systems of  
record

FHIR Server    CQL Engine    UM Platform    CM Platform    Integration Engine  
MDM    IDP

#### LAYER 03

##### Foundation

Cloud, data & trust  
infrastructure

Hosting    Infrastructure    Networking    Data Warehousing    Security    Privacy

Your stack is your stack. We extend what you have. Where there are gaps, we plug them with the right partner, chosen for fit, not margin.

### HIKE HEALTH® • PLATFORM PHILOSOPHY

#### THE CBH LENS • ON-PREM IS NOT A BLOCKER

CBH's existing claims, UM, and care-management platforms stay, on-premises. The hybrid pattern fits: **systems of record remain on-prem; the FHIR foundation and public API surface run in a cloud landing zone CBH owns**, fed through the integration layer. Public endpoints get cloud-grade uptime, identity, and scale; PHI governance and Part 2 consent stay under CBH control instead of inside a vendor's portal stack. That is what makes the rating-period timeline achievable without a core-systems replacement program.



PART V Proof

14 · CASE STUDY · COMMUNITY HEALTH PLANS OF WASHINGTON

HIKE HEALTH® · FIRELY · CHPW · EHEALTH EXCHANGE

## A dQM pilot built on a real data acquisition program.

A Medicaid-focused payer piloting NCQA digital quality measures on FHIR + CQL, with a parallel data acquisition workstream to surface missing clinical data, the closest analogue in this guide to CBH's own footprint.

### CHALLENGE

#### Quality data, but in pieces

Clinical data scattered across provider feeds and abstraction vendors; legacy HEDIS stack unable to execute CQL.

### APPROACH

#### FHIR + CQL + acquisition

Firely FHIR R4 store, USCDI-aligned ·  
CQL engine for high-volume measures ·  
HIKE-led data acquisition from  
QHINs, HIEs, and provider networks ·  
TEFCA connectivity · parallel reporting  
alongside legacy HEDIS.

### OUTCOME

#### Continuous, complete, reusable

Continuous in-year measure  
performance · clinical data  
completeness materially improved ·  
reusable foundation for full HEDIS  
migration.

## Highlight metrics

4

#### DQM MEASURES PILOTED

high-volume clinical measures

Increased %

#### DATA COMPLETENESS GAIN

vs claims-only baseline

Continuous

#### MEASUREMENT CADENCE

replacing annual chart-chase

The data acquisition workstream is the part that made dQM real. The FHIR stack alone would not have been enough.

CHPW

### WHY THIS MATTERS TO CBH

A Medicaid plan, a community provider network, and quality measures starved for clinical data: CHPW's starting point is CBH's. The same playbook applies directly to FUH, IET, and the BH measure set.





## 15 · CASE STUDY · JEFFERSON HEALTH PLANS

HIKE HEALTH® · AZURE HEALTH DATA SERVICES · JEFFERSON HEALTH PLANS

### ePA workflows, brought to a single operating model.

A regional payer (Philadelphia-based, like CBH) modernizing prior authorization on FHIR Da Vinci PAS, with HIKE HEALTH as operating partner and Azure Health Data Services as the FHIR platform.

#### CHALLENGE

##### Manual PA at scale

Fax-and-portal workflows, long decision turnaround, and pending CMS-0057-F filings with no operating model in place.

#### APPROACH

##### PAS + CRD/DTR + governance

Azure Health Data Services FHIR R4 + Da Vinci PAS stood up · CRD/DTR integration with UM workflow · HIKE-led governance & vendor coordination · provider pilot with three EHR partners.

#### OUTCOME

##### Days to minutes

ePA decision time from days to minutes for routine PAs · Mar 31, 2026 filing on a defined operating model · foundation reused for drug ePA.

### Highlight metrics

#### days → mins

##### EPA DECISION TIME

routine PA paths, structured submissions

#### 3

##### EHR PARTNERS

in initial provider pilot

#### 1

##### OPERATING MODEL

spanning medical and drug ePA

HIKE HEALTH® acted as our operating partner, not as a vendor handing us a build and walking away. That is the difference.

JEFFERSON HEALTH PLANS

#### WHY THIS MATTERS TO CBH

Same city, same provider community, same deadline pressure. The UM-integration and provider-pilot patterns proven at JHP translate to CBH's BH service lines, with consent and provider enablement layered in.



PART VI Ecosystem & Partners

16 · TECHNOLOGY PARTNERS & HOW TO START

## The ecosystem we climb with.

HIKE HEALTH® is vendor-agnostic by design. We co-lead the standards bodies that publish the rules, and we partner with the technology vendors that implement them.

| CATEGORY               | PARTNERS                                                                                                                       | ROLE                                  |
|------------------------|--------------------------------------------------------------------------------------------------------------------------------|---------------------------------------|
| FHIR Servers           | Smile · Firely · Aidbox · Medplum · Epic Payer Platform · Azure Health Data Services · AWS HealthLake · Google Healthcare APIs | FHIR R4 platforms                     |
| Integration            | Mirth Connect · Rhapsody · Redox · BridgeLink                                                                                  | HL7v2 ↔ FHIR bridging                 |
| Identity & Trust       | Auth0 · OneLogin · ID.me · Okta · Clear · Microsoft Entra ID                                                                   | SMART/OAuth, IAL2 proofing            |
| Consent & Segmentation | Consent2Share-pattern tooling · FHIR Consent · security-labeling approaches for Part 2 data                                    | 42 CFR Part 2 enforcement             |
| MDM & Data             | Verato · NextGate · 4Medica · Snowflake · Databricks                                                                           | Member-record matching, analytics     |
| Networks               | TEFCA QHINs · Carequality · CommonWell · HSX (HealthShare Exchange) & regional HIEs                                            | Cross-payer / cross-provider data     |
| Cloud                  | AWS · Microsoft Azure · Google Cloud                                                                                           | Hosting, security, scale              |
| Standards Bodies       | HL7 Da Vinci · HL7 FAST · NCQA · ONC · Civitas Networks                                                                        | HIKE HEALTH® contributors & co-chairs |

### THE CBH LENS

**HSX is CBH's home-field advantage.** Philadelphia's regional HIE already connects the health systems CBH members use. A data acquisition strategy that starts with HSX and TEFCA reaches the clinical data BH quality measures need, without point-to-point provider integrations.

### START HERE

#### Start with a 30-minute conversation.

Bring your roadmap. We'll show you where one operating model replaces five programs, and what the first two quarters look like for CBH. [info@hike.health](mailto:info@hike.health) · (732) 470-0235 · [hike.health](https://hike.health)



APPENDIX The CBH Map

17 · THE FIRST TWELVE MONTHS

## A sequenced readiness map, anchored to the rating period.

Working backward from the first rating period beginning on or after January 1, 2027, the date the Prior Authorization, Provider Access, and Payer-to-Payer APIs must be live.

| WINDOW                | WORKSTREAM                                                                                                                                                                                                                                                    | DECISION / DELIVERABLE                                           |
|-----------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------|
| <b>Q3 2026</b><br>now | Applicability memo & gap assessment vs 0057-F operational provisions · PA metrics baseline · inventory of PA volume by BH service line · 9115-F health check on the HealthTrio (mPulse) stack, incl. Part 2 segmentation                                      | Executive sponsor named · one program backlog stood up           |
| <b>Q4 2026</b>        | FHIR platform & hosting decision (facade vs durable server · cloud landing zone for the on-prem estate) · Da Vinci PAS / CRD / DTR architecture · consent-management design · HealthTrio (mPulse) extend-or-replace decision · UM-platform integration design | Architecture locked · build funded · vendors contracted          |
| <b>Q1 2027</b>        | Build & end-to-end testing · provider-enablement pilot with top-volume BH providers · runbooks, monitoring, audit packaging · metrics instrumentation for the Mar 31 filing cycle                                                                             | APIs live per rating period · pilot providers submitting via PAS |
| <b>Q2 2027</b>        | Run-state SLAs · expand ePA to residential/IOP concurrent review · 0053-F attachments planning (May 2028 horizon) · dQM pilot scoping on FUH & IET                                                                                                            | First operational quarter reviewed · dQM pilot funded            |

### The owner's bench, mapped to CBH's leadership structure

| PROGRAM ROLE                                | NATURAL HOME AT CBH                                                                                                               |
|---------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------|
| <b>Executive sponsor</b>                    | Office of the CEO · Donna E.M. Bailey, MBA, MSEd                                                                                  |
| <b>Platform, APIs &amp; integration</b>     | CIO Arshad Hussain · Information Technology (Samuel Wright)                                                                       |
| <b>Data, dQM &amp; analytics</b>            | Chief Data & Research Officer Suet Lim, PhD · Data Analytics (Aelesia Piscielli, PhD)                                             |
| <b>UM criteria &amp; clinical policy</b>    | CMO Samuel L. Williams III, MD · Clinical Innovation & Utilization (Lauren DellaCava) · Quality Management (Michele Kidwell Kane) |
| <b>Provider enablement &amp; operations</b> | COO Andrew Devos · Provider Operations (Kimberly Doyle) · Member Services (Noelle Ciara)                                          |
| <b>Compliance, audit &amp; filings</b>      | Program Integrity / Compliance Officer Ken Inness · General Counsel Melissa Pavlow                                                |
| <b>Pharmacy intersection (0062-P watch)</b> | Pharmacy Initiatives (Oluwatoyin Fadeyibi, PharmD, MPH)                                                                           |
| <b>Funding &amp; vendor contracts</b>       | CFO Alex Gauthier · Project Management (Brooke Stello)                                                                            |



**HIKE HEALTH®**

Health Information Knowledge  
Exchange

EDITION 2026 · PREPARED FOR CBH

THE OPERATING MODEL IS THE DIFFERENCE

# Let's climb together.

When the regulations land, the payers that finish on schedule will be the ones that treated this as one operating-model upgrade, not parallel scrambles. For Philadelphia's behavioral health safety net, the stakes are ~780,000 members. That is the work HIKE HEALTH® does.

## STRATEGY

Program design across the CMS rules and NCQA dQM, framed for executive sponsors.

## EXECUTION

FHIR build, integration, ePA workflows, consent, identity, and end-to-end testing.

## RUN-STATE

Managed operations with clear SLAs across your platforms and vendors.

## START A 30-MINUTE CONVERSATION

hike.health · info@hike.health  
(732) 470-0235 · Philadelphia, PA



SCAN · HIKE.HEALTH